

Penn Presbyterian Medical Center

 MD, F.A.C.C.
Director, Interventional Cardiac Electrophysiology

December 31, 2024

RE: DOB: MRN: Dear 

I had the pleasure of seeing your patient, , in Arrhythmia Clinic at our Penn Cardiology offices at Cherry Hill. As you know, the patient is an 82-year-old gentleman with a history of hypertension, hyperlipidemia, AAA, untreated obstructive sleep apnea, COPD/asthma, back pain, transient global amnesia in 2020, diastolic CHF, chronic left anterior fascicular block/right bundle branch block, and a high burden of atrial ectopy who returns for arrhythmia management.

Self-monitoring through his Apple watch suggested a 10% burden of possible paroxysmal atrial fibrillation. Strips were not reviewable, and systemic anticoagulation was deferred pending confirmation. A recent 30 day monitor documented very high 22.2% burden of premature atrial contractions but no sustained atrial fibrillation.

He admits to intermittent episodes of unexplained shortness of breath and dizziness, but denied any severe palpitations or loss of consciousness. He had no complaint of chest discomfort, orthopnea, or PND.

His biggest complaint has been lower back pain despite lumbar spine surgery by Dr. Osturk 2-1/2 years ago. He ran out of gabapentin, and asked me to provide him with a short-term prescription. He was also investigating possible acupuncture for additional pain relief.

He has worn off alcohol use. He may have an occasional cup of coffee. He admitted to noncompliance of CPAP for sleep apnea.

Medical History:

- 1) Hypertension
- 2) hyperlipidemia
- 3) DOE (dyspnea on exertion)
- 4) Abnormal EKG
- 5) AAA (abdominal aortic aneurysm)
- 6) Prediabetes
- 7) OSA (obstructive sleep apnea)
- 8) COPD with asthma (CMS-HCC)
- 9) Supraventricular premature beats
- 10) Ascending aorta dilatation (CMS-HCC)

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- 11) Edema of right lower extremity
- 12) Near syncope
- 13) PAF (paroxysmal atrial fibrillation)
- 14) TGA (transient global amnesia) - 5/2020
- 15) Cardiomyopathy (CMS-HCC)
- 16) Coronary artery calcification seen on CT scan - 1/2024

Family History:

Stroke: Sister (at age 74 yo)

CAD: Negative History

There is no family history of sudden cardiac arrest.

Social History:

He reports that he quit smoking about 13 years ago. His smoking use included cigarettes. He started smoking about 53 years ago. He has a 30 pack-year smoking history. He has never been exposed to tobacco smoke. He has never used smokeless tobacco. He reports current alcohol use of about 28.0 standard drinks of alcohol per week. He reports that he does not use drugs.

Review of Systems:

Respiratory: Positive for shortness of breath.

Cardiovascular: Positive for palpitations.

Musculoskeletal: Positive for back pain and joint pain.

All other systems reviewed and are negative.

Allergy List:

- 1) Bee Pollen
- 2) Dust
- 3) Grass
- 4) Lisinopril - Cough - Other reaction(s): Cough Cough
- 5) Mixed Ragweed - Other reaction(s): Unknown Other reaction(s): Unknown
- 6) Pollen [Tree Pollen]
- 7) Seasonal

Medications (reflects new prescriptions):

- 1) Acetaminophen (tylenol extra strength) 500 mg tablet, take 2 tablets by mouth every 6 hours as needed (mild pain).
- 2) Alpha-lipoic acid 600 mg po caps, take 1 capsule by mouth daily.
- 3) Aspirin 81 mg ec tablet, take 1 tablet by mouth daily.
- 4) Cholecalciferol (vitamin d3) 1.25 mg (50000 ut) capsule, take 1 capsule by mouth once a week.
- 5) Dronedarone 400 mg tabs tablet, take 1 tablet by mouth 2 times a day.
- 6) Famotidine 20 mg tablet, take 1 tablet by mouth 2 times a day.
- 7) Fluticasone 50 mcg/act nasal spray, administer 2 sprays into both nostrils daily as needed.
- 8) Gabapentin 300 mg capsule, take 1 capsule by mouth every 12 hours.
- 9) Losartan 25 mg tablet, take 1 tablet by mouth daily.
- 10) Meloxicam 15 mg oral tablet disintegrating, take by mouth.
- 11) Methylcobalamin (methyl b-12 po), take 1 tablet by mouth daily.
- 12) Metoprolol succinate er 25 mg tablet, take 0.5 tablets by mouth daily.
- 13) Montelukast 10 mg tablet, take 1 tablet by mouth daily at bedtime.
- 14) Rosuvastatin 20 mg tablet, take 1 tablet by mouth daily.

15) Spironolactone 25 mg tablet, take 0.5 tablets by mouth daily.

Physical Exam:

A comprehensive cardiovascular examination was performed.

The blood pressure, pulse, respirations and weight for this visit were as follows:

BP 106/73 (BP Location: left upper arm, Patient Position: Sitting, Cuff Size: Regular) | Pulse (!) 105 |
Ht 5' 11" (1.803 m) | Wt 193 lb (87.5 kg) | SpO2 92% | BMI 26.92 kg/m²

General: well-developed, well-nourished, and no acute distress

HEENT: normal conjunctiva, mucus membranes moist, and sclera anicteric

Chest: clear to auscultation, normal diaphragmatic excursion

Heart: regular rhythm, normal S1, normal S2, no murmurs, and no rubs or gallops

Neck: normal jugular venous pressure, negative Kussmaul's, carotids normal volume without bruits, no thyromegaly

Vascular: peripheral pulses intact and no groin hematomas or bruits

Abd: normal active bowel sounds, soft, non-tender, and non-distended

Ext: tr bilateral lower extremity edema

Neuro: alert, oriented x 3, grossly nonfocal

Skin: no rash or cellulitis

12-lead EKG:

Twelve lead EKG 12/31/24 documented sinus rhythm with Mobitz 1 second-degree AV block, left axis deviation consistent with left anterior fascicular block, QRS 132 in a right bundle branch block pattern, and QTC 441 milliseconds. There was poor R-wave progression. There were nonspecific T-wave abnormalities.

Twelve lead EKG 09/06/2024 documented sinus rhythm at a rate of 67 beats per minute, left axis deviation consistent with left anterior fascicular block., PR 312 consistent with first-degree AV block, QRS 140 in a right bundle branch block pattern, QTC 454 milliseconds. There was delayed R-wave progression. There were no ischemic ST-T abnormalities.

Cardiac Testing:

24 hour Holter 04/12/2023:

1) Normal sinus rhythm with heart rate ranging from 61 to 126 and average heart rate of 88 BPM.

2) There were no pauses over 2 seconds. There was underlying first-degree AV block and an intraventricular conduction defect. There were occasional junctional escape beats albeit not at a slow rate.

3) There were 12,761 supraventricular premature beats (10.3% of all beats), 103 couplets, and 115 runs of atrial tachycardia. The longest episode of atrial tachycardia lasting 85 beats with a maximum heart rate of 112 beats per minute. The fastest episode of atrial tachycardia lasted 3 beats with a maximum heart rate of 156 beats per minute. There was no atrial fibrillation.

4) There were 231 ventricular premature beats (0.2% of all beats), 3 couplets, 1 triplets, and no ventricular tachycardia.

5) There were no symptomatic episodes reported.

Pharmacologic stress nuclear imaging 04/28/2023:

- A perfusion 1-day rest/stress protocol was performed on a conventional

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SPECT camera.

- A pharmacologic stress test was performed using regadenoson.
- There were no significant ECG changes during vasodilator stress.
- The stress end diastolic cavity size is normal.
- Global left ventricular function during stress is normal. The stress imaging left ventricular ejection fraction is 52%. There was no evidence of ischemia.

Chest CTA 01/08/2024 at Cooper Medical Center:

LOWER NECK: Normal.

HEART: Chambers are normal size. No pericardial effusion is present.

Coronary artery calcifications

VESSELS: Atheromatous calcifications in the aorta and origins the great vessels. Soft plaque within the mid to distal thoracic aorta. Enlargement of the main pulmonary artery up to 3.6 cm, suggestive of pulmonary retention.

MEDIASTINUM: There is no mediastinal hematoma or pneumomediastinum.

LUNGS, AIRWAYS AND PLEURAL SPACE: Mild emphysematous changes in the lungs. No pleural effusion. No pneumothorax. Dependent changes in both lung bases.

BONES: Multilevel degenerative changes in the spine. Diffuse osteopenia.

CHEST WALL AND SOFT TISSUE: Sebaceous cysts noted within the anterior chest wall measuring 1.9 cm.

TTE 5/16/24:

- The left ventricle is normal in size. There is mildly increased left ventricular wall thickness consistent with mild concentric hypertrophy. LV IVS measures 1.2 cm. There are no segmental wall motion abnormalities. Normal left ventricular ejection fraction. The left ventricular ejection fraction by biplane method of discs is 56%. The average global longitudinal peak systolic strain is borderline. The average global longitudinal peak systolic strain is 17.6% (absolute value).
- Grade III (severe) LV diastolic dysfunction.
- The right ventricle is normal in size. There is normal function of the right ventricle.
- The left atrium is mildly dilated.
- The right atrium is normal in size.
- The mitral valve is normal in structure. There is mild mitral annular calcification. There is mild mitral regurgitation. There is no mitral stenosis.
- The aortic valve is trileaflet. There is mild aortic valve thickening. There is mild aortic valve calcification. There is mildly reduced excursion of the aortic valve. There is no aortic stenosis. There is no aortic regurgitation.
- The tricuspid valve is normal in structure. There is mild tricuspid regurgitation. Pulmonary artery systolic pressure measures 32 mmHg. The pulmonary artery systolic pressure is normal.
- The aortic root is enlarged. The aorta measures 4.3 cm at the sinus of

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Valsalva. The aorta measures measures 2.2 cm/m2 at the sinus of Valsalva, indexed for body surface area. The proximal segment of the ascending aorta is enlarged. The proximal segment of the ascending aorta measures 4.0 cm. The proximal segment of the ascending aorta measures 2.0 cm/m2, indexed for body surface area.

- The inferior vena cava is dilated (diameter >21 mm) and decreases >50% in size with inspiration, suggesting an elevated right atrial pressure of 8 mmHg (range 5-10 mmHg).
- Compared to the prior study of 4/6/2023, LV systolic function is now normal, but diastolic dysfunction is noted. The aortic root/ascending aorta are similar in size.

Cardiac Event Monitoring: 09/23/2024 through 10/23/2024

1. The predominant rhythm was sinus rhythm with heart rates between 33 and 127 beats per minute. The average rate was 70 beats per minute.
2. There were very frequent (22.2% burden) supraventricular beats. There was a 4 beat run of nonsustained atrial tachycardia
3. There were occasional premature ventricular beats. There was no nonsustained or sustained ventricular tachycardia.
4. There was no atrial fibrillation.
5. There was Mobitz type 1 second-degree AV block. There were no significant pauses.
6. There were no patient diary entries.

Labs:

Sodium:	12/1/2023: Sodium, Serum 145 4/4/2024: Sodium, Serum 144
Potassium:	12/1/2023: Potassium, Serum 3.5 4/4/2024: Potassium, Serum 4.6
Chloride:	12/1/2023: Chloride, Serum 102 4/4/2024: Chloride, Serum 106
Co2:	12/1/2023: Carbon Dioxide, Total 21 4/4/2024: Carbon Dioxide, Total 21
BUN:	12/1/2023: BUN 13 4/4/2024: BUN 10
Creatinine:	12/1/2023: Creatinine, Serum 0.74 4/4/2024: Creatinine, Serum 0.63
Glucose:	12/1/2023: Glucose, Serum 106 4/4/2024: Glucose, Serum 88
Cholesterol:	1/31/2023: Cholesterol, Total 168 12/1/2023: Cholesterol, Total 143
LDL:	No results
LDL Direct::	No results
HDL:	No results
Triglyceride:	No results
AST:	1/31/2023: AST (SGOT) 25 12/1/2023: AST (SGOT) 26
ALT:	1/31/2023: ALT (SGPT) 20 12/1/2023: ALT (SGPT) 20

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Digoxin:	No results
WBC:	3/14/2022: White Blood Cells 6.7 3/15/2022: White Blood Cells 8.9
Hgb:	No results
Platelets:	3/14/2022: Platelets 368 3/15/2022: Platelets 333
Hgb A1C:	6/17/2021: Hemoglobin A1c 5.8 4/1/2022: Hemoglobin A1c 6.5
TSH:	5/10/2020: TSH 2.620 12/1/2023: TSH 0.719
INR:	3/14/2022: INR 1.0 3/15/2022: INR 1.0

Orders Placed This Encounter

- aspirin 81 mg EC tablet
- CMP (prior to your next visit)
- dronedarone 400 MG TABS tablet
- EKG [93000] (performed in office) ECG
- gabapentin 300 MG capsule

IMPRESSION:

- 1) Premature atrial contractions, high burden
- 2) R/O PAF (paroxysmal atrial fibrillation) (CMS-HCC)
- 3) Transient global amnesia vs TIA
- 4) Right bundle branch block (RBBB) with left anterior fascicular block (LAFB)
- 5) Chronic diastolic congestive heart failure (CMS-HCC)
- 6) Coronary artery calcification seen on CT scan - 1/2024
- 7) Abdominal aortic aneurysm (AAA) without rupture, unspecified part (CMS-HCC)
- 8) COPD with asthma (CMS-HCC)
- 9) OSA (obstructive sleep apnea), noncompliant with CPAP
- 10) Essential hypertension

Your patient presents with a high >22% burden of atrial ectopy. He has had no confirmed episodes of atrial fibrillation, but would potentially be at risk for progression to sustained atrial fibrillation.

He had a previous mechanical fall that resulted in a right hip fracture, so we elected not to start systemic anticoagulation. This fall appears to be an isolated event that was associated with alcohol intoxication, so we agreed to anticoagulation for any sustained atrial tachyarrhythmias.

He agreed to start a baby aspirin at 81 milligrams daily. He wished to pursue a trial of Multaq 400 milligrams twice daily for PAC suppression. If unaffordable, then a possible switch to amiodarone would be a low cost alternative.

He was provided with a laboratory slip for a comprehensive metabolic panel be obtained approximately 1 to 2 weeks after antiarrhythmic drug initiation. He wished to undergo event recorder implantation to allow for earliest possible detection of any recurrent sustained atrial arrhythmias that would prompt re-initiation of an anticoagulant.

We had a frank discussion regarding the limitations of this strategy due to inefficiencies related to data

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transmission, arrhythmia interpretation, and patient communication. However, he was willing to accept these risks, and elected to proceed with implantation of a Medtronic LINQ event recorder. Our schedulers will plan to contact him at 856-466-4708.

I agreed to provide him with a 1 month supply of gabapentin at 300 milligrams twice daily. He plans to get refills from his usual provider. He has an appointment to see Dr. Jong Shin for a trial of acupuncture.

I emphasized the importance of compliance with CPAP for sleep apnea. He will plan to discuss the matter further with sleep medicine. He has a pre-existing appointment with Dr. Joffe in about a month.

I spent a total of 40 minutes preparing to see the patient, obtaining and reviewing separately obtained history, performing medically appropriate examination or evaluation, counseling and educating patient/family/caregiver, ordering medications, tests or procedures, referring and communicating with other health care professionals and documenting clinical information in electronic or other record on the day of the visit. There was no time overlap with separately billable procedures such as electrocardiograms, event monitoring, or device interrogations.

Thank you for allowing me to participate in the care of your patient. Please do not hesitate to contact me with any questions or concerns.

With special regards,